

D5b — Psychiatry Consult Note

Dr. L. Okafor, Psychiatry | Psychiatry | 02/19/2026 14:30

Patient: Pemberton, Harold J. **MRN:** 847-29-3156 **DOB:** 04/12/1951 **Allergies:** Sulfa drugs (rash)

PSYCHIATRY CONSULT NOTE

Patient: Harold J. Pemberton | MRN: 847-29-3156
Date: 02/19/2026 14:30 | Provider: Dr. L. Okafor, Psychiatry (PGY-4)
Reason for Consult: Depression management – SSRI discontinued during admission

HISTORY:
Mr. Pemberton has a history of Major Depressive Disorder, well-controlled on sertraline 100 mg daily for approximately 3 years. Sertraline was discontinued on HD #1 by the admitting team due to concern for hyponatremia risk in the setting of acute illness, IV fluid administration, and the potential for aggressive diuresis if CHF decompensation is confirmed.

Patient reports stable mood. Denies suicidal ideation, hopelessness, or anhedonia at present. Acknowledges he "depends on that medication" and is worried about being off it.

PHQ-9 score today: 8 (mild)

ASSESSMENT:
MDD, currently in partial remission on medication. Agree that sertraline poses hyponatremia risk in this clinical context (acute illness, volume shifts, potential diuretic use). Abrupt discontinuation of sertraline after 3 years of use is suboptimal but clinically necessary in this setting.

RECOMMENDATIONS:

- Do not restart sertraline. Transition to bupropion is intentional.
- START bupropion XL 150 mg PO daily
 - No hyponatremia risk
 - Safe in CHF (no QTc prolongation concern at this dose)
 - Safe in COPD
 - Additional benefit: bupropion has activating properties which may help with fatigue/low energy
- Follow-up outpatient psychiatry in 2-4 weeks for dose optimization (may increase to 300 mg)
- Monitor mood during hospitalization. If acute worsening, re-consult.

Electronically signed by: Dr. L. Okafor, MD (PGY-4)

Cosigned by: Dr. A. Brennan, MD (Attending, Psychiatry)

Date: 02/19/2026 15:00

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